

Baillargeon et al. v. Murray et al.

[Indexed as: Baillargeon v. Murray]

52 O.R. (3d) 278
[2001] O.J. No. 148
Court File No. 596/95

Ontario Superior Court of Justice
Henderson J.
January 15, 2001

Damages--Personal injuries--Deductions--Plaintiff settling his claim against his no-fault insurer for lump sum--Defendants claiming that plaintiff made improvident settlement and that they were entitled to have plaintiff's damages for past wage loss reduced under s. 267(1)(a) of Insurance Act by amount to which he was entitled rather than by amount which he received --Defendants proving on balance of probabilities that additional no-fault benefits were available to plaintiff --Defendants entitled to have additional amount deducted from damages for past wage loss--Insurance Act, R.S.O. 1990, c. I.8, s. 267(1)(a).

Damages--Personal injuries--Loss of future income--Plaintiff suffering serious injuries to cervical and lumbar spine in three motor vehicle accidents--Damages for future loss of income assessed at \$97,630.

Damages--Personal injuries--Pain and suffering--Plaintiff suffering serious injuries to cervical and lumbar spine in three motor vehicle accidents--General damages assessed at \$110,000.

Insurance--Automobile insurance--No-fault regime--Threshold requirements--Plaintiff injured in two motor vehicle accidents

under Ontario Motorists Protection Plan amendments to Insurance Act and third motor vehicle accident under Bill 164 amendments to Insurance Act--Global assessment of plaintiff's injuries in first two accidents putting his condition over Ontario Motorists Protection Plan threshold--Both tortfeasors contributing materially to plaintiff's global injuries--Neither tortfeasor immune from liability--Third accident significantly aggravating plaintiff's injuries--Third accident materially contributing to plaintiff's condition--Third accident causing injuries which exceeded Bill 164 threshold--Third tortfeasor not immune from liability.

The plaintiff was injured in three motor vehicle accidents. The first two accidents were governed by the Ontario Motorists Protection Plan (O.M.P.P.) amendments to the Insurance Act, and the third accident was governed by the Bill 164 amendments to the Insurance Act. As a result of the first accident, which occurred on April 24, 1993, when the plaintiff was 55 years old, the plaintiff suffered a fracture of the C-6 vertebra and a musculoligamentous injury to his neck. The accident also caused an acceleration of degenerative changes in the plaintiff's cervical spine and lumbar spine which had been asymptomatic before the accident. In the second motor vehicle accident, which occurred on May 25, 1993, the plaintiff was jostled and struck his head on the door frame of his vehicle. The accident caused an aggravation of the initial injuries to the plaintiff's cervical spine and lumbar spine and further accelerated the degenerative changes in the cervical spine and the lumbar spine. The contribution of the second motor vehicle accident to the plaintiff's overall problems was significant. The third motor vehicle accident aggravated the initial injuries to the cervical spine and the lumbar spine and continued the acceleration of the degenerative disc disease commenced by the first accident. Myelomalacia of the spinal cord commenced before the third accident, but was significantly aggravated by the third accident. The third accident made a significant contribution to the plaintiff's overall problems, and the problems caused by the third accident became intermingled with the problems caused by the first two accidents. The plaintiff suffered from bowel and bladder problems which were caused by a combination of all three

accidents since the problems related to the spinal cord. He also had an ongoing problem of sexual dysfunction as a result of a combination of the three accidents. The plaintiff and his wife brought an action for damages against the drivers of all three vehicles that were involved in the accidents. Liability for the accidents was admitted.

Held, the action should be allowed.

The thresholds under the Ontario Motorists Protection Plan amendments to the Insurance Act and the Bill 164 amendments to the Insurance Act were different. Under the former provisions, tortfeasors are immune from liability unless the injured person has died or has sustained permanent serious disfigurement or permanent serious impairment of an important bodily function caused by continuing injury which is physical in nature. The Bill 164 threshold provides a blanket immunity from a finding of liability in respect of pecuniary or non-pecuniary loss, and then provides a specific exception in the case of non-pecuniary loss where the injured person has died or has sustained serious disfigurement or serious impairment of an important physical, mental or psychological function. In the case of multiple tortfeasors with a single threshold, if a plaintiff suffers from injuries which, globally assessed, put his condition over the threshold, then the tortfeasors who materially contributed to the plaintiff's global condition are to be found to have caused injuries which exceeded the threshold, and are therefore not to be immune from liability by reason of the protection afforded by the Insurance Act. Where there is more than one threshold involved, the plaintiff's damages must be assessed at appropriate times after each motor vehicle accident in order to determine the combined effect of the accidents up to that point.

The first accident caused a serious and permanent impairment of an important bodily function caused by continuing injury which was physical in nature. The plaintiff's injuries exceeded the O.M.P.P. threshold. The combined effects of the first and second accidents were permanent and serious. After the second accident, the plaintiff's condition exceeded the O.M.P.P. threshold. Both accidents materially contributed to the

plaintiff's condition as it existed prior to the third accident. Therefore, both accidents caused an impairment that exceeded the O.M.P.P. threshold. After the third accident, the plaintiff's impairment was well in excess of the Bill 164 threshold. There is no requirement for permanence of injury under Bill 164, but the fact that the plaintiff had suffered a permanent injury could be taken into account when considering whether or not the injury was "serious". There was no doubt that the plaintiff's injuries were serious after the third accident. The third accident significantly aggravated the plaintiff's injuries. Therefore, it materially contributed to the plaintiff's condition. The third accident caused injuries which exceeded the Bill 164 threshold. As the tortfeasors in all three accidents caused injuries that exceeded their respective thresholds, subject to the other restrictions set out in the Insurance Act, all of the defendants were responsible for the plaintiff's damages.

General damages were assessed against the defendant in the first accident at \$66,000; against the defendant in the second accident at \$22,000; and against the defendant in the third accident at \$22,000.

The plaintiff would not be gainfully employed for the balance of his life. Given that he had been working in the heavy construction industry for his entire working life, that he had significant, although asymptomatic, degenerative disease throughout his spine, and that he wanted to spend time with his wife, who retired early, the plaintiff would not have worked after the age of 65. Damages for past income loss, without any deduction for no-fault benefits, were assessed at \$304,483.

Damages for future income loss were assessed at \$97,630. As the plaintiff was totally disabled as a result of the first accident, the defendants in the second and third accidents were not liable for any of that amount.

Section 267(1)(a) of the O.M.P.P. states that damages awarded to a plaintiff are to be reduced by "all payments that the person has received or that were or are available for statutory accident benefits and by the present value of any statutory

accident benefits to which the person is entitled". The plaintiff settled his claim against his no-fault insurer by accepting a full and final lump sum payment as consideration for any no-fault benefits that the insurer would have been required to pay to the plaintiff then or in the future. The weekly income benefits which the plaintiff received and the lump sum totalled \$110,855. The defendants took the position that the plaintiff had available to him a further sum of \$87,511 for weekly income benefits that he should have received up to the date of the trial, and submitted that, because the plaintiff made an improvident settlement with his insurer, they should get credit for this further sum against the past lost income award pursuant to s. 267(1)(a). In order to succeed in that claim, the defendants were required to prove on a balance of probabilities that the no-fault benefits "were or are available" to the plaintiff. The defendants provided sufficient evidence for that finding to be made. Therefore, pursuant to s. 267(1)(a), the past lost income award was to be reduced by the sum of \$110,855 actually received by the plaintiff, and by the further sum of \$87,511 which the plaintiff had available to him.

It was beyond dispute that the plaintiff would not engage in any form of employment whatsoever between now and age 65. Therefore, the present value of future no-fault benefits should be deducted from the future lost income award. It would be illogical to deduct no-fault benefits payable for life from a future loss of income award that was capped at age 65. No-fault benefits beyond age 65 are not the same "type of loss" akin to a future loss of income award to age 65. Therefore, the future loss of income award should be reduced by the present value of future no-fault benefits to which the plaintiff was entitled from the date of trial to age 65.

The plaintiff's wife had lived with him for 16 years. They were married in 1992. Before the accidents, they were very active together. As a result of the plaintiff's injuries, his wife felt that she had lost the companionship of the man that she married. The plaintiff had developed a rambling style of conversation and frequently made no sense. He was short-tempered and sexually dysfunctional. The wife's Family Law Act, R.S.O. 1990, c. F.3 damages were assessed at \$7,000.

Alderson v. Callaghan (1998), 40 O.R. (3d) 136, 42 C.C.L.T. (2d) 230, 21 C.P.C. (4th) 224 (C.A.); Athey v. Leonati, [1996] 3 S.C.R. 458, 140 D.L.R. (4th) 235, 203 N.R. 36, [1997] 1 W.W.R. 97, 31 C.C.L.T. (2d) 113, apld

Bannon v. McNeely (1998), 38 O.R. (3d) 659, 159 D.L.R. (4th) 223, 34 M.V.R. (3d) 189 (C.A.) (sub nom. Bannon v. Hagerman Estate); Berns v. Campbell (1974), 8 O.R. (2d) 680, 59 D.L.R. (3d) 44 (H.C.J.); Chrappa v. Ohm (1998), 38 O.R. (3d) 651, 159 D.L.R. (4th) 215, 33 M.V.R. (3d) 284 (C.A.); Hicks v. Cooper (1973), 1 O.R. (2d) 221, 41 D.L.R. (3d) 454 (C.A.); Hiscox v. Badger, [1979] O.J. No. 15, consd

Other cases referred to

Cattapan v. Mitchell (1978), 27 O.R. (2d) 87, 105 D.L.R. (3d) 508, [1980] I.L.R. 1-1254 (S.C.); Collee v. Kyriacou (1996), 31 O.R. (3d) 558, 25 M.V.R. (3d) 288 (Gen. Div.); Dalmatin v. Raposo (2000), 46 O.R. (3d) 749 (S.C.J.); McGhee v. National Coal Board, [1972] 3 All E.R. 1008, [1973] 1 W.L.R. 1, 116 Sol. Jo. 967 (H.L.); Meyer v. Bright (1993), 15 O.R. (3d) 129, 110 D.L.R. (4th) 354, 48 M.V.R. (2d) 1 (C.A.) [Leave to appeal to S.C.C. refused (1994), 17 O.R. (3d) xvi, 172 N.R. 160n]; Orchover v. Wright (1996), 28 O.R. (3d) 263, 23 M.V.R. (3d) 201 (Gen. Div.); Stante v. Boudreau (1980), 29 O.R. (2d) 1, 112 D.L.R. (3d) 172, [1980] I.L.R. 1-1250 (C.A.)

Statutes referred to

Family Law Act, R.S.O. 1990, c. F.3

Insurance Act, R.S.O. 1970, c. 224, s. 237(2)

Insurance Act, R.S.O. 1990, c. I.8, s. 266(1), 266(1)(b)

Insurance Statute Law Amendment Act, 1993, S.O. 1993, c. 10 (Bill 164), ss. 267(1), 267(1)(a)-(c), 267.1(1), 267.1(2), 267.1(8)

ACTIONS for damages for personal injuries.

Mason Greenaway and Elizabeth Essex, for plaintiffs.

W.H. Peter Madorin, Q.C., for defendants, Francis Murray and Mary J. Murray.

Michael T. Mollison, Q.C., for defendant, M. Barbara Kirk.

Brian Wagner, for defendant, Ganesh Sukdeo.

[1] HENDERSON J.:--The plaintiffs are husband and wife. They bring this action for damages as a result of three separate motor vehicle accidents in which the plaintiff John Baillargeon was injured. (Hereinafter, where I refer to the plaintiff in the singular form, I am referring to John Baillargeon.)

[2] The defendants are the owners and operators of the three motor vehicles that were responsible for the three motor vehicle accidents. The first and second motor vehicle accidents, which occurred on April 24, 1993 and May 25, 1993 respectively, are covered by the Ontario Motorists Protection Plan amendments to the Insurance Act, R.S.O. 1990, c. I.8, and the third motor vehicle accident, which occurred on December 7, 1994, is covered by the Bill 164 [now S.O. 1993, c. 10] amendments to the Insurance Act.

Issues

[3] Liability for the motor vehicle accident of April 24, 1993 (MVA #1) has been admitted by the defendants Murray. Similarly, liability for the motor vehicle accident of May 25, 1993 (MVA #2) and the motor vehicle accident of December 7, 1994 (MVA #3) has been admitted by the defendants Kirk and Sukdeo respectively.

[4] Therefore, I am being requested by the plaintiffs to assess their damages against all three defendants. John Baillargeon claims general damages, past loss of income, future loss of income and future care costs. Cecile Baillargeon claims under the Family Law Act, R.S.O. 1990, c. F.3, as a result of the injuries suffered by John Baillargeon. At first blush this should be a relatively simple matter.

[5] However, the claims are complicated by the fact that there are three separate motor vehicle accidents, each of which caused or exacerbated certain injuries. I must make findings of fact as to which motor vehicle accident caused which injuries, and the resultant effects of each accident on the plaintiffs.

[6] The assessment of damages is further complicated by the legislated requirements set out in the Insurance Act. This becomes more difficult because of the fact that the first two MVAs occurred under one insurance regime, and the third MVA occurred under another insurance regime.

[7] The case is even further complicated by the fact that the defendants are entitled to have the damages awarded by this court reduced by all no-fault benefits that John Baillargeon has received or that were available, and by the present value of any no-fault benefits to which he is entitled. The plaintiff John Baillargeon settled his claim against his no-fault insurer, Zurich Insurance Company, by accepting a full and final lump sum payment as consideration for any no-fault benefits that Zurich would have been required to pay to the plaintiff now or in the future. The defendants take the position that the amount that the plaintiff received from Zurich Insurance Company in settlement of this claim was too low, and therefore the defendants claim credit for the amount which the defendants say the plaintiff was entitled to receive from Zurich Insurance Company.

[8] In order to decide this case I must therefore answer the following questions:

1. What medical injuries or conditions were caused by each of the three MVAs?
2. Do the medical injuries or conditions caused by each of the MVAs meet or exceed the thresholds set out in the relevant insurance regimes?

If any of the injuries or conditions caused by the three MVAs do not meet the relevant threshold, then the plaintiffs' claims with respect to that MVA should be dismissed.

If any of the MVAs caused injuries or conditions which exceed the relevant threshold, then I must answer the following questions:

3. At what amount do I assess the general damages of the plaintiff John Baillargeon for each of the MVAs?
4. How do I apply the deductible amount that is set out in Bill 164 to the plaintiff's general damages?
5. At what amount do I assess the plaintiff's past loss of income for each of MVA #1 and MVA #2? (Pecuniary damages are prohibited by Bill 164 and therefore do not apply to MVA #3.)
6. At what amount do I assess the plaintiff's future lost income for each of MVA #1 and MVA #2?
7. What amount is to be deducted from the plaintiff's damages for past lost income and future lost income in order to give the defendants credit for no-fault benefits which John Baillargeon received, had available, or was entitled to receive from Zurich Insurance Company?
8. At what amount do I assess the future care costs of the plaintiff John Baillargeon for each of MVA #1 and MVA #2?
9. At what amount do I assess the Family Law Act claim of the plaintiff Cecile Baillargeon for each of the MVAs, and how do I apply the deductible amount set out in Bill 164?

Background

[9] John Baillargeon was born on March 11, 1938. He was 55 years of age at the time of the first motor vehicle accident and is currently 62 years of age. He left high school in grade 10 and joined the Canadian Armed Forces, where he served for approximately three years. During his time in the Canadian Armed Forces he completed courses which allowed him to achieve a grade 12 education.

[10] The plaintiff Cecile Baillargeon is John Baillargeon's third wife. Mr. Baillargeon has four children with his first wife and two children with his second wife. All of the children are grown adults and make no claim in this action. Mr. Baillargeon met Cecile Baillargeon in 1984. They lived together for approximately eight years, and then married in 1992. They have now lived together for the last 16 years. They have no children with each other.

[11] Initially, the plaintiffs lived together in Windsor, but moved to Kitchener in approximately 1985 to allow John Baillargeon to take employment with Marcon Custom Metals Inc. They moved back to Windsor together in October 1996, and have remained in Windsor since that time.

[12] John Baillargeon has a long work history with many employers. From the late 1950s until approximately 1985, he worked for many different employers, primarily as a welder/fitter. His longest period of continuous employment with one employer was approximately one and one-half years. Despite the considerable number of employers, it appears that Mr. Baillargeon was regularly employed. Mr. Baillargeon testified that his longest period of unemployment during this time was approximately one month, and that was his own choice as he had worked many hours during that particular year.

[13] John Baillargeon's services appear to have been in high demand. He has no formal diploma or certificate as a welder/fitter, but learned the trade on the job. He took short-term jobs on a contract basis, and he was generally able to obtain a new contract whenever an old contract ended.

[14] Mr. Baillargeon added to his skills by learning to be an underwater diver/welder. He would dive in a hard-hat outfit or in scuba gear to underwater projects where he would weld or salvage as the job demanded. This additional skill probably made Mr. Baillargeon's services more desirable.

[15] In approximately 1985 Mr. Baillargeon took a job with Marcon Custom Metals Inc. as a layout fitter and welder. His

duties involved constructing items from drawings. He started with drawings of the item which was to be constructed, and he formulated all the steps in order to build the item from the drawings. This included the allocation of machinery and personnel to the job. He said that he took this job because he wanted an everyday job that he could do with little or no travelling. He wanted to lead a normal life, sleep in his own bed, and settle down with his wife. Mr. Baillargeon continued to work at the Marcon job until the first motor vehicle accident.

[16] Mr. Baillargeon had some exotic hobbies during his early years, including skydiving, car racing and motorcycle racing. He admittedly stopped these activities well before the first motor vehicle accident.

[17] Mr. Baillargeon also built models of boats and airplanes as a young man. Initially he built these models from kits, but his hobby evolved to the point where he would obtain drawings of certain boats and build scale models of the boats from the actual drawings. This was similar to the kind of work that he did as a layout fitter. He continued to do this activity until his motor vehicle accidents. Sometime after the MVAs he resumed this activity and still does it today.

[18] Also, it should be noted that Mr. Baillargeon has involved himself in an exercise program since he was a young man. This program involved the use of weights and machines, and at times included running and cycling. He worked out regularly prior to MVA #1. Thereafter, after his therapy program ceased, he developed his own workout program, which he still follows to date, although the program is limited because of his injuries.

[19] Prior to the motor vehicle accident, the plaintiff John Baillargeon stated that he was a very healthy and active person. He had a good relationship with his wife. He had a good job, and he felt that he performed his work well. He had an active exercise program, and he engaged in the hobby of building scale models from drawings. This state of affairs was corroborated by Cecile Baillargeon.

Credibility of John Baillargeon

[20] John Baillargeon was a pedantic and overly talkative witness. It was very difficult for the questioner to obtain a short answer from Mr. Baillargeon as Mr. Baillargeon always wanted to give a lengthy background narrative before answering any question. I find that this habit of Mr. Baillargeon's was not for the purpose of evading the question, but for the purpose, in Mr. Baillargeon's mind, of attempting to ensure that his answers were fully explained.

[21] Unfortunately, because of this tendency, some of the answers that he gave were not clear. Often he explained certain answers by giving background that related to time periods that had absolutely no relationship to the questions that were being asked of him. Therefore, it was often difficult to determine his exact answer. On many occasions, by the time he came around to giving his answer, I am certain that he had forgotten the question.

[22] Furthermore, there were clearly several contradictions between the testimony given by Mr. Baillargeon at trial and other statements made by Mr. Baillargeon either at his examinations for discovery, or to his doctors, or in certain documents. In fact, there were obvious contradictions within the testimony that Mr. Baillargeon gave at trial. Most of these contradictions concerned the timing of the onset of certain symptoms, and the existence of certain symptoms at specific points in time.

[23] Dr. Macartney-Filgate (a psychologist) conducted psychological testing on Mr. Baillargeon in June 2000. As a result of her testing, Dr. Macartney-Filgate concluded that Mr. Baillargeon was a man who appears to have a high need for control and has placed a lot of value on his physical functioning. She felt that his physical outlets had served as an important means of dealing with his psychological or emotional conflicts or stresses prior to the MVAs. As a result of the motor vehicle accidents, Mr. Baillargeon had a dysfunctional pattern of somatic preoccupation and somatic hypervigilance with increased needs and attempts at exercise of

control over others. That is, he had a high need for control and was preoccupied with his injuries.

[24] I clearly observed on the witness stand Mr. Baillargeon's need for control and his dysfunctional pattern of somatic preoccupation. It was these traits that made it difficult to get a direct short answer from Mr. Baillargeon. I believe that John Baillargeon now looks back on the last seven years since MVA #1 and sees a myriad of physical problems through which he suffered. Because of his preoccupation with these problems he is now unable to accurately tell us precisely how he felt at certain times, yet he is unable to admit his inability to do so.

[25] I find that Mr. Baillargeon was not attempting to mislead the court, but I have a great deal of difficulty relying on the accuracy of his evidence with respect to the timing of his symptoms, and the degree of severity of his symptoms. I find that I can rely on John Baillargeon's evidence on general matters, and on some traumatic unforgettable matters, but for specific information I must be guided by the medical records.

MVA #1--April 24, 1993

[26] At the time of MVA #1 John Baillargeon was alone in his Nissan Micra motor vehicle, driving southbound on Victoria Street in Kitchener at approximately 30 miles per hour. His seatbelt was fastened, as it was for all of the motor vehicle accidents in question.

[27] The Murray vehicle struck Mr. Baillargeon's vehicle from behind at a high rate of speed. The Baillargeon vehicle was pushed by the impact to and through an intersection, and came to rest on the south side of the intersection a "fair distance" away from the point of impact.

[28] The driver's seat of the Baillargeon vehicle broke as a result of the impact and Mr. Baillargeon ended up in the backseat of his vehicle with his body twisted around, his arm draped over the backseat, and his feet still in the front of

the vehicle tangled in his seatbelt. The Baillargeon vehicle was towed from the scene of the accident and was written off by his insurance company. Mr. Baillargeon was driven home by the police officer who attended the scene, but within one hour took a cab to St. Mary's Hospital.

[29] Mr. Baillargeon testified that he initially suffered injuries in the form of headaches, neck pain, low back pain and right shoulder pain. Later, but before the second accident, he developed numbness on the right side of his face from a point near his right ear and flowing into his right cheekbone.

[30] In between MVA #1 and MVA #2, Mr. Baillargeon saw Dr. Yan (his family doctor) and commenced taking physiotherapy at the Canadian Back Institute at the request of Dr. Yan and Zurich Insurance Company (John Baillargeon's insurance company).

[31] Just prior to MVA #2, he was still going to the Canadian Back Institute five times per week on a rehab program, which was never completed before MVA #2. He made no attempt to return to work before MVA #2.

MVA #2--May 25, 1993

[32] In MVA #2, John Baillargeon was driving his vehicle in a parking lot at Highland Hills Mall in Kitchener when his vehicle was struck by the front end of the Kirk vehicle, which was moving out of a parking spot. Again, the plaintiff had his seatbelt fastened. The plaintiff said that he struck the left side of his head on the door frame of his vehicle as a result of the impact. He did not lose consciousness but was stunned. This triggered pain in his neck and head immediately. He was taken from the scene of the accident to St. Mary's Hospital complaining of headaches and neck pain.

[33] After MVA #2, the plaintiff was again treated by his family doctor, Dr. Yan, but also started seeing several specialists, including Dr. Elfeki (neurosurgeon) and Dr. Bambrick (psychologist). He returned to Canadian Back Institute and continued his previous rehabilitation program.

[34] After MVA #2, and after some treatment, the plaintiff made two unsuccessful attempts to return to work. The first attempt was made in September 1993, at the suggestion of Zurich Insurance Company, or their agents, despite the plaintiff's opinion that he could not do his former job. The return to work was made on a part-time basis, five hours per day, four days per week. After 28 1/2 hours on the job, Mr. Baillargeon was unable to continue.

[35] The second return to work attempt was made in January 1994, and again was on the basis of five hours per day, four days per week. Again, the return to work was done at the suggestion of Zurich Insurance Company, or their agents, and was done despite Mr. Baillargeon's opinion that he could not do his job. This attempt to work lasted four or five weeks, at which time Mr. Baillargeon was unable to continue. There has been no attempt to return to work since January 1994.

[36] After the unsuccessful attempts to return to work, John Baillargeon continued to see Dr. Yan. He complained to Dr. Yan of right shoulder pain, some neck pain, some headaches, and low back pain with symptoms radiating into both thighs and down the right leg. He also testified that he began to develop problems with bowel and bladder incontinence and with sexual dysfunction.

MVA #3--December 7, 1994

[37] In MVA #3, the plaintiff was a front seat passenger, with his seatbelt fastened, in a vehicle which was stopped in the curb lane on Westmount Road in Kitchener beside a Volvo which was stopped in the passing lane. When the light turned green the Sukdeo vehicle entered the intersection on a red light from the left of the Volvo. The Sukdeo vehicle struck the Volvo and caused the Volvo to strike the plaintiff vehicle. Mr. Baillargeon testified that the driver's side of the plaintiff vehicle was lifted up and dropped down again, causing the plaintiff's body to move first toward the driver of his vehicle and then back towards the passenger side of the vehicle. His body did not hit anything within the plaintiff vehicle and

there was no loss of consciousness.

[38] Immediately after the motor vehicle accident, the plaintiff felt a really sharp pain in his low back, and then felt electric surges from his low back down both legs and into his testes. Thereafter, he had a complete loss of feeling from the waist down. He remained seated in the plaintiff vehicle after the impact. Gradually, he started to get some of the feeling back in his lower body.

[39] The plaintiff said that immediately after MVA #3 he felt that his headaches were a little worse in severity and the pain frequency increased in his neck. He also felt that his facial numbness was more frequent and had more depth.

[40] The plaintiff felt that his low back was much worse after MVA #3. Prior to the MVA #3, he felt that he had burning in both thighs, but he only had symptoms down his right leg. After MVA #3, he had the same burning in his thighs but he had pain and numbness down both his left and his right leg. His feet were cold and numb on both sides.

[41] Mr. Baillargeon went on to say that he developed a left shoulder problem that radiated down his arm to his left hand. He first noticed this in late 1996 and it reached its worst point about four or five months before the surgery he had in September 1998. After the September 1998 surgery, he started physiotherapy on the left shoulder and after approximately two months of physiotherapy the shoulder felt much better.

[42] After the plaintiffs moved to Windsor in approximately October 1996, John Baillargeon started seeing another set of doctors. He saw Dr. Carom (family doctor) who made several referrals to specialists. He saw Dr. Chakravarthi (neurosurgeon) who performed two surgeries. The first surgery was September 10, 1998, and was a C-5/6 vertebrectomy, C-4/5/6/7 discectomy and cervical fusion at C-4/5/6/7. This surgery reduced Mr. Baillargeon's neck pain and headaches, and further reduced the facial numbness. It was after this surgery that Mr. Baillargeon had physiotherapy on the frozen left shoulder.

[43] The second surgery was a L-4/5 laminectomy, foraminotomy, and lumbar fusion at L-4/5 and S-1, and took place on October 18, 1999. This surgery significantly reduced the plaintiff's back pain and reduced the shooting pains in his legs.

[44] John Baillargeon has not attempted to return to any form of employment. Neither Dr. Carom nor Dr. Chakravarthi believe that the plaintiff will return to any form of gainful employment.

Causation of Injuries

a. Neck and back injuries

[45] Prior to MVA #1, John Baillargeon did not have any ongoing musculoskeletal or neurological problem. Dr. Yan was Mr. Baillargeon's family doctor for approximately five years prior to MVA #1. Dr. Yan stated that Mr. Baillargeon had problems with blood in his stool, and an ulcer in his stomach area, prior to MVA #1. John Baillargeon specifically did not have any neck, back, head, shoulder or leg problems in at least the five years prior to MVA #1 during which Dr. Yan was his family doctor.

MVA #1

[46] After MVA #1 John Baillargeon testified that he initially complained of injuries in the form of headaches, neck pain, low back pain and right shoulder pain. Later, but before the second accident, he developed numbness in the right side of his face. The St. Mary's General Hospital record of April 24, 1993 shows that Mr. Baillargeon complained of problems with his neck, forehead, right lower back, right side of his face, right shoulder and scapula and right hip. Dr. Yan first saw Mr. Baillargeon after MVA #1 on April 26, 1993, at which time Mr. Baillargeon complained of pain in his neck, low back and the right side of his head. Dr. Yan at that time noted that there was spasm in the plaintiff's paracervical muscles and sent Mr. Baillargeon for an X-ray of his cervical spine.

[47] An X-ray of the cervical spine was done on April 26, 1993 and the radiology report described a narrowing of the disc space at C-4/5 and C-5/6 with osteophytes and encroachment in the foramina. This report did not seem to contain any other findings that were significant to Dr. Yan. Dr. Yan saw Mr. Baillargeon again on April 27 and April 30, 1993, and on both of those occasions Mr. Baillargeon complained of neck and low back pain. Dr. Yan prescribed some medication and gave Mr. Baillargeon some advice with respect to neck exercises.

[48] The April 26, 1993 X-ray takes on more significance when I consider the evidence of Dr. Chakravarthi. Dr. Chakravarthi, on a referral from Dr. Carom, performed the cervical surgery on September 10, 1998. I found Dr. Chakravarthi to be an impressive witness. He was clearly knowledgeable in the field of neurosurgery, and he was fair and direct in his testimony.

[49] When Dr. Chakravarthi conducted the cervical surgery he noticed that the C-6 vertebra was soft and extremely vascular, which indicated to him that the C-6 vertebra had been fractured in the past.

[50] Dr. Chakravarthi then reviewed the X-ray films (not the radiology reports) of the cervical spine taken in April 1993 and May 1993, immediately after MVA #1 and MVA #2 respectively. He observed from the films that there was a reduced height of the C-6 vertebra in both films. Therefore, it was Dr. Chakravarthi's belief that the 1993 X-rays showed a fracture at C-6 which was not recognized by either of the radiologists at the time.

[51] Dr. Chakravarthi therefore concluded that the trauma of MVA #1 caused the C-6 fracture. Dr. Carom accepted this opinion.

[52] Therefore, I find as a fact that as a result of MVA #1, John Baillargeon suffered a fracture of the C-6 vertebra and associated cervical problems, which never completely healed, and which continued to cause him problems up until his surgery by Dr. Chakravarthi, and thereafter.

[53] It is also clear that the plaintiff suffered from a musculoligamentous injury to his neck as a result of MVA #1. This was described as a neck strain by Dr. Yan, and later was called a cervical spine strain by Dr. McKnight (physiatrist). By November 1994, Dr. Iwan (neurosurgeon) called it a soft tissue injury.

[54] Furthermore, the 1993 X-rays showed degenerative disc disease at C-4/5 and C-5/6. Dr. Chakravarthi conceded that the degenerative disc disease was present before MVA #1, although it was [asymptomatic]. Dr. Chakravarthi also conceded that the degenerative disc disease was a progressive condition, and may have become symptomatic in the absence of any motor vehicle accidents or other trauma, although the rate of progression and the severity of the symptoms would be impossible to predict.

[55] It was Dr. Chakarvarthi's opinion, and I so find, that MVA #1, in addition to causing a fracture at C-6 and a soft tissue injury to the neck, caused an acceleration of the degenerative changes in the cervical spine. I find that at least some of the symptoms that the plaintiff had with his neck were because of the degenerative changes in the cervical spine which were accelerated and made symptomatic by MVA #1.

[56] Also, Dr. Chakravarthi reviewed an MRI in June 1998 which showed myelomalacia at C-4/5/6/7. This is a permanent bruising of the spinal cord as a result of an injury which has caused some of the myelin surrounding the spinal cord to be lost. Dr. Chakravarthi said that the myelomalacia was secondary to the degenerative changes in the cervical spine. Also, a myelogram done June 12, 1998, showed significant indentation of the spinal cord at C-3 to C-6.

[57] It was Dr. Chakravarthi's opinion, and I so find, that the degenerative process which led to the indentation of the spinal cord and the myelomalacia was triggered or accelerated by MVA #1 and aggravated by MVA #2 and #3.

[58] Dr. Chakravarthi and Dr. Carom testified that the neck injuries suffered by the plaintiff could cause neck pain,

headaches, shoulder pain and arm and hand problems. The injury at C-4/5 could also cause facial numbness around the ear and into the jaw.

[59] Regarding the low back symptoms, there was an X-ray of the lumbar spine done May 6, 1993 which showed mild degenerative disc disease from L-1 through to L-5. No narrowing of the disc space or encroachment in the foramina was noted.

[60] In June 1998, Dr. Chakravarthi reviewed a CT study and myelogram which showed advanced degenerative disc disease at L-4/5, disc herniation and narrowing at the neural foramina. Dr. Chakravarthi felt, and I so find, that the trauma of MVA #1 also accelerated the degenerative changes in the low back, which were previously asymptomatic. The acceleration of the degenerative changes in the low back were aggravated by MVA #2 and MVA #3. These degenerative changes could cause pain, numbness and weakness into the hips and down the legs to the feet. There was also a lumbar soft tissue injury as noted by Dr. Yan, Dr. McKnight and Dr. Iwan.

[61] Dr. Yan's clinical notes show that just before MVA #2, Mr. Baillargeon was continuing to complain of neck pain, low back pain, and pain in the right side of his face. There was one occasion on which pain was noted to radiate into the right thigh.

[62] Therefore, I find that immediately before MVA #2, Mr. Baillargeon was still suffering from pain associated with the fractured C-6 vertebra, pain associated with a soft tissue injury to the neck and low back, facial numbness on the right side of his face, and a radiation of pain from the low back into the right thigh. I also find that MVA #1 caused the acceleration of asymptomatic degenerative changes in the cervical spine and lumbar spine, and that the effects of these degenerative changes were starting to cause pain to the plaintiff. I find that Mr. Baillargeon had not recovered to any significant degree from any of these injuries prior to MVA #2.

MVA #2

[63] At the accident scene after MVA #2, the plaintiff complained of head and neck pain. He had hit his head on the door frame and was stunned. This is corroborated by Ms. Kirk, who testified that she took a cigarette out of the plaintiff's fingers as he seemed distraught, and did not seem to notice the cigarette. She thought that he might burn himself.

[64] According to the ambulance call report, the plaintiff told the ambulance attendant that his headache was worse and his neck pain was worse. At the emergency room, the records show that the plaintiff complained of pain in the neck and a headache.

[65] The plaintiff testified that his headaches went from a seven or eight out of 10 on the pain scale before MVA #2, to 10 out of 10 on the pain scale immediately after MVA #2. Similarly, he testified that the pain in his neck went from an eight or nine out of 10 before MVA #2 to 10 out of 10 after MVA #2. He testified that his low back was a little worse after MVA #2, but then settled back down. He did not note any other changes as a result of MVA #2.

[66] Dr. Yan's notes show that there were several complaints of headache, neck pain and right shoulder pain after MVA #2 and through to December 1993. In cross-examination John Baillargeon acknowledged that the head and neck pain subsided, or resolved, by late 1993 or early 1994.

[67] There is substantial support for the proposition that the plaintiff's neck problems continued, at least in some form, until MVA #3. The plaintiff complained of neck and shoulder problems through 1993 and into 1994 to Dr. Elfeki, Dr. McKnight, the therapist who conducted a Functional Capacities Evaluation (FCE), Dr. Moore and Dr. Israel, the latest complaint being in July 1994 to Dr. Moore. By November 17, 1994, the plaintiff told Dr. Iwan that his problems were with the low back, right hip and right leg. However, Dr. Iwan still noted that the plaintiff's range of motion in the neck was decreased on that day.

[68] Dr. Chakravarthi testified that the acceleration of the

degenerative disc disease by its nature will cause periods of ebb and flow. That is, there will be times at which the plaintiff feels better, and times at which he feels worse. Both Dr. Carom and Dr. Chakravarthi testified that if the plaintiff was feeling better for a certain period of time, or had no symptoms for a certain period of time, this did not mean that the underlying problem had gone away. Therefore, I find that although the plaintiff may have had few complaints of headaches, shoulder pain or neck pain in the months leading up to the motor vehicle accident in December 1994, the underlying problems caused by MVA #1 and #2 had not gone away.

[69] The plaintiff also continued to have a significant low back problem after MVA #2. This is recorded in the notes and records of Dr. Yan. His first complaint of a low back problem to Dr. Yan following MVA #2 was made June 7, 1993. Thereafter, there were approximately 24 visits to Dr. Yan prior to MVA #3 at which the plaintiff complained of low back pain, or Dr. Yan noted stiffness or reduced range of motion in the low back. The plaintiff thought that his low back had settled down somewhat after MVA #2, but worsened significantly in approximately August 1994.

[70] Dr. Yan's notes show that there were complaints of radiation of pain from the low back into both thighs and down the right leg. These complaints were noted on at least three occasions in 1994 prior to MVA #3.

[71] Significant complaints of low back problems are noted in various specialists' reports throughout 1993 and 1994. He complained of a low back problem to Dr. Elfeki as early as June 22, 1993. Complaints to Drs. MacCon, McKnight, Israel and Moore continued into the summer of 1994. When Dr. Iwan saw the plaintiff on November 17, 1994, he was complaining of pain in the low back, right hip and right foot. Dr. Iwan noted a reduced range of motion in the [plaintiff's] back at that time.

[72] As of November 1994, it was Dr. Iwan's opinion that the neck and low back problems were caused by these MVAs. He did not distinguish between the two MVAs. Dr. Israel felt that the symptoms to the head, neck, right shoulder and low back

increased after MVA #2. Dr MacCon felt that MVA #2 exacerbated the head, neck and back pain.

[73] It was Dr. Chakravarthi's opinion that MVA #1 caused most of the plaintiff's injuries, and that the subsequent accidents aggravated the symptoms. He does not distinguish between the contribution of MVA #2 and #3.

[74] A well-established legal maxim is that the tortfeasor takes his victim as he finds him. This is sometimes referred to as the thin-skull doctrine. At the time of MVA #2, the plaintiff was suffering from a fractured vertebra at C-6, a musculoligamentous injury to the cervical spine and lumbar spine, and an acceleration of the degenerative changes in the cervical spine and the lumbar spine. He was also starting to have pain as a result of damage to his spinal cord. He was thus extremely vulnerable to injury from any trauma.

[75] Unfortunately for all concerned, Ms. Kirk committed an act of negligence, which has been admitted, which inflicted some trauma on the plaintiff in MVA #2. There is no doubt that the plaintiff was at least jostled in his vehicle as a result of the impact in MVA #2, and I find as a fact that he did strike his head on the door frame inside his vehicle. Therefore, even though the impact may not have been severe in MVA #2, the effects on the plaintiff were significant and Ms. Kirk is responsible for the consequences of her act.

[76] I therefore find as a fact that MVA #2 caused an aggravation of the initial injuries to the cervical spine and lumbar spine, and further accelerated the degenerative changes in the cervical spine and the lumbar spine. The contribution of MVA #2 to the overall problems was significant. MVA #2 essentially contributed to the deterioration of the same parts of the body that were injured by MVA #1. The effect of MVA #2 had not resolved by the time of MVA #3.

MVA #3

[77] MVA #3 caused the most dramatic effects of any of the motor vehicle accidents to the plaintiff. Immediately after MVA

#3, the plaintiff said that he had electrical surges through his lower body and thereafter felt numb from the waist down. As he sat in the automobile, he gradually started to get the feeling back in his legs.

[78] Dr. Chakravarthi testified that the electrical surges in the lower limbs indicated a problem with the spinal cord at C-4/5/6 in the cervical spine. There was evidence that at least one incident occurred prior to MVA #3 in which the plaintiff suffered from electrical surges in his lower limbs. This occurred in early 1994 when an elevator dropped suddenly and as a result, the plaintiff was jarred and suffered surges in his lower body. Dr. Chakravarthi confirmed that this incident showed that there was a problem with the spinal cord in the cervical spine prior to MVA #3.

[79] The ambulance call report after MVA #3 confirmed complaints of right leg numbness, right lumbar pain, left leg burning and no movement in the right foot. The emergency room report also indicated worse symptoms in the lower back and legs after MVA #3.

[80] Immediately after MVA #3, Dr. Yan's notes indicate that he diagnosed a mid-back injury caused by MVA #3. In the plaintiff's first attendance on Dr. Yan after MVA #3, the plaintiff complained of headaches, pain in the right chin and jaw, shoulder pain, hip pain and tenderness in the upper and lower spine.

[81] Thereafter, Dr. Yan's notes show that there were several complaints of radiation of pain and numbness in both legs. A review of Dr. Yan's notes indicates that the radiation of pain into both legs was much more prominent after MVA #3. Also, there were clearly more complaints of radiation into the left leg after MVA #3 than before MVA #3.

[82] By the time the plaintiff saw Dr. Carom for the first time on February 4, 1997, he was complaining of neck pain, low back pain, left arm and shoulder stiffness, numbness in the right side of his face and pain and burning in both legs.

[83] Dr. Carom made several referrals to specialists and arranged for several diagnostic tests. This led to the plaintiff undergoing MRIs and myelograms, which were reviewed by Dr. Chakravarthi, as mentioned earlier. Also, it should be noted that a CT scan of the lumbar spine on September 26, 1997 showed that a disc fragment at L-5/S-1 had broken off of the disc and moved upwards in the spine.

[84] I find that all of the plaintiff's symptoms with respect to the neck, low back and both legs did not abate, but appeared to worsen, after MVA #3. Eventually they necessitated surgery.

[85] Dr. Chakravarthi was of the opinion, and I so find, that MVA #3 aggravated the initial injuries to the cervical spine and lumbar spine, and continued the acceleration of the degenerative disc disease commenced by MVA #1, all of which had been aggravated by MVA #2. I find that the myelomalacia of the spinal cord commenced before MVA #3, but was significantly aggravated by MVA #3.

[86] I also find that MVA #3 made a significant contribution to the overall problems of the plaintiff, and the problems caused by MVA #3 became intermingled with the problems caused by MVA #1 and #2.

[87] Regarding the surgeries, I find as a fact that both surgeries were necessitated by a combination of MVA #1, #2 and #3. Again, Dr. Chakravarthi testified, and I so find, that most of the symptoms were related to MVA #1, and were aggravated by MVA #2 and #3.

b. Shoulder and arm problems

[88] At times, the plaintiff had symptoms of pain, numbness and weakness in both shoulders and down both arms to the hands. I find that these complaints were as a result of the cervical spine injuries suffered in MVA #1 and aggravated by MVA #2 and #3.

[89] The right shoulder problems started with MVA #1. During the time between MVA #2 and MVA #3 the plaintiff made many

complaints of pain in his right shoulder to Dr. Yan. Dissatisfied by the results he was getting from his treating physicians, the plaintiff privately retained the services of Dr. Shaman in approximately September 1993. He received treatment from Dr. Shaman until approximately the end of October 1993. By the time he left Dr. Shaman's treatment program, the plaintiff testified that he was feeling much better in his right shoulder. He felt that his right shoulder healed by the end of 1993 and was never a problem thereafter.

[90] The plaintiff developed problems in both hands and down both arms over time. These problems were alleviated by the neck surgery. There are still some minor problems with the hands.

[91] The only part of the shoulders or arms that is questionably related to the accidents is the left shoulder. The plaintiff testified that in the fall of 1996, he commenced having stiffness in his left shoulder. This developed to the point where he had a frozen shoulder. The frozen shoulder was confirmed by Dr. Carom in 1997. The plaintiff received some treatment to the left shoulder after the neck surgery, which caused a significant improvement to the left shoulder, although the problem has not completely resolved.

[92] Dr. Carom was of the opinion that the left shoulder problem was as a result of degenerative changes in the cervical spine. He felt that the plaintiff had C-5 radiculopathy, which affected the shoulder area. In fact, he noted that the left deltoid muscle was weak, and this confirmed his diagnosis.

[93] It was Dr. Carom's opinion that the left shoulder problem was caused by the MVAs as the MVAs started the acceleration of the degenerative process. I therefore find as fact that the left shoulder problem, as well as symptoms in the right shoulder and both hands and arms, were caused by a combination of all three MVAs.

c. Facial numbness

[94] I find as a fact that the facial numbness was caused by an impingement of the nerve root in the cervical spine as a

result of the cervical injury, which itself was caused by MVA #1 and aggravated by MVA #2 and #3.

[95] The facial numbness was substantially improved by the neck surgery, and the plaintiff is left with a little light numbness in the side of his face.

d. Leg symptoms

[96] After MVA #1, the plaintiff complained to Dr. Yan of pain radiating from the low back to the right leg on one occasion. After MVA #2 and before MVA #3 the plaintiff made several complaints of numbness and burning in both thighs, and radiation of pain into the right leg, to Dr. Yan. He also complained of radiation into the right hip and right foot to Dr. Iwan in November 1994.

[97] After MVA #3, the plaintiff initially complained to Dr. Yan on December 8, 1994 of pain in the right hip, and at that time Dr. Yan also noticed that the plaintiff was limping. By January 1995, the plaintiff complained to Dr. Iwan that he had a dulling sensation from the waist down. In fact, Dr. Yan confirmed on March 7, 1995 that the plaintiff was limping and both legs were numb.

[98] I find that there were some complaints of leg symptoms after MVA #1, and these increased after MVA #2, and they further increased after MVA #3. After MVA #3, the plaintiff was walking with a limp and had burning in both thighs and radiation of pain down both legs, as opposed to only one leg.

[99] Dr. Chakravarthi testified that the leg complaints were caused by an injury to the low back, which was suffered in MVA #1 and aggravated by MVA #2 and #3. I therefore find that all of the leg symptoms were caused by the low back injury suffered in MVA #1 and aggravated by MVA #2 and #3. I also find that the leg complaints worsened after each MVA.

e. Numbness and coldness in the feet

[100] The plaintiff testified that he had numbness and

coldness in the feet at least since MVA #3. He felt this was related to the motor vehicle accidents.

[101] Dr. Carom sent the plaintiff to Dr. Ianicello (vascular surgeon), who was of the opinion that the plaintiff's problem was not vascular in nature, and therefore he felt that the problem was neurological.

[102] Dr. Carom therefore felt that the plaintiff had a condition known as peripheral neuropathy. There are many causes for peripheral neuropathy, including diabetes or lack of vitamin B-12. It is also known that this condition can occur spontaneously without any known cause. There was no evidence whatsoever that this condition was caused by the motor vehicle accidents. Therefore, I find that the numbness and the cold feet of the plaintiff was not caused by any of the MVAs.

f. Left foot problem

[103] The plaintiff had a callus on the left foot removed surgically by Dr. Bernstein. The plaintiff felt that he developed this callus because of a peculiar gait that he developed after the MVAs.

[104] Dr. Bernstein in his report said that the callus was due to the splaying of the forefoot. Dr. Carom confirmed this opinion, and specifically felt that this problem was due to fallen arches as a result of the aging process. Therefore, I find that the left foot problem was not related to any of the MVAs.

g. Bladder and bowel problems

[105] The plaintiff testified that he had bladder and bowel incontinence problems, particularly after MVA #3. He felt that he may have had some episodes prior to MVA #3, but did not pay much attention to them. After MVA #3 he commenced wearing pads because he found that he was dribbling. This condition was substantially improved by the low back surgery in 1999.

[106] Dr. Carom had the plaintiff seen by both Dr. Sharma

(neurologist) and Dr. Pearce (urologist) for this problem. The plaintiff told both doctors that his problems with dribbling started after MVA #3.

[107] Dr. Sharma suspected that the bladder problem was caused by a spinal cord stretch or contusion. Dr. Pearce found that the problems were not vascular. Dr. Carom testified that the motor vehicle accident trauma probably contributed to the bladder problems. He thought that the plaintiff may have a condition called a neurogenic bladder, caused by the MVAs. Dr. Chakravarthi thought that the bowel and bladder problems could be caused by the myelomalacia.

[108] Defence counsel submitted that it appears that the bladder problems started after the hemorrhoid surgery that the plaintiff underwent in March 1995. After the surgery, he had problems voiding and had a catheter installed for two or three days. However, I find that the problem voiding after surgery was not the same problem as the incontinence problem.

[109] Therefore, I find that the bowel and bladder problems were caused by a combination of MVA #1, #2 and #3, as the problems relate to an injury to the spinal cord. The etiology is not precisely known, but on a balance of probabilities, the bladder and bowel problems were neurogenic in nature and were caused by a combination of all three MVAs.

h. Sexual dysfunction

[110] The plaintiff complained that he had a sexual dysfunction as a result of these motor vehicle accidents. He stated that he initially could not have any sexual relationship with his wife after MVA #1 and #2 because of pain. Sometime thereafter he attempted to have sexual intercourse but had difficulty first achieving and then sustaining an erection.

[111] On his examination for discovery, the plaintiff testified that he had an excellent sex life until he started taking Prozac in May 1997. This is in direct contradiction to his evidence at trial.

[112] Dr. Carom testified that he became aware of the sexual dysfunction problem on July 2, 1997 when the plaintiff complained that the Prozac that Dr. Carom had prescribed one month earlier was causing sexual problems. Dr. Carom testified that Prozac is known to cause sexual dysfunction. Prozac had been prescribed for the plaintiff since May 1997, as a result of the plaintiff's depression, which arose from the stress of the motor vehicle accidents.

[113] Therefore, I make a finding of fact that aside from an initial period after MVA #1 and #2 during which the plaintiff was unable to have sexual relations because of pain, any sexual dysfunction thereafter did not arise until July 1997. Thereafter, I find that the plaintiff has had an ongoing problem with sexual dysfunction, which continues to date. The ongoing problem is as a result of a combination of the three motor vehicle accidents.

i. Head injury and psychological injuries

[114] The plaintiff's doctors considered whether or not the plaintiff had suffered a mild head injury in MVA #1 and MVA #2. Concentration and memory problems were noticed after MVA #2 by Dr. Elfeki. His wife testified that Mr. Baillargeon developed a rambling conversational style, in which he would lose track of what he was saying, after MVA #3.

[115] The plaintiff complained of head pain after all three accidents. Dr. Chakravarthi explained that head pain can be caused by a concussion, musculoligamentous injury or referred pain from the disc injury. In this situation Mr. Baillargeon probably had head pain from a combination of all three sources. I note that very few complaints of head pain were actually made by the plaintiff after MVA #3.

[116] Both Dr. Goldberg and Dr. Macartney-Filgate gave written opinions that the plaintiff may have suffered from a mild closed head injury in MVA #1 and/or MVA #2, but he had recovered fully from the effects of the head injury by the time of their examinations in 1995 and 2000, respectively.

[117] Dr. Goldberg specifically stated in his report of November 7, 1995 that the plaintiff's test results were consistent with a mild closed head injury that had mainly resolved to pre-accident levels.

[118] However, both Dr. Goldberg and Dr. Macartney-Filgate felt that the plaintiff continued to show evidence of psychological and emotional problems. Dr. Goldberg, in 1995, said that the plaintiff had an Affective Disorder with episodes of depression and hypomania.

[119] Dr. Macartney-Filgate said that the plaintiff had become destabilized as a result of the motor vehicle accidents. She said that the plaintiff had a dysfunctional pattern of somatic preoccupation and somatic hypervigilance. This destabilization most likely disrupted the plaintiff's previous coping style and sense of self. She said that Mr. Baillargeon has had emotional and psychological difficulties as a result of his accidents.

[120] Therefore, I conclude that a combination of MVA #1 and #2 caused a closed head injury which resolved at least by 1995. I also find that MVA #1, #2 and #3 combined to cause ongoing emotional and psychological difficulties for the plaintiff which have not resolved. The fact that Mrs. Baillargeon testified that the rambling conversational style did not start until after MVA #3 only indicates that this problem continued to get worse with each MVA. The additional stress of MVA #3 in all likelihood caused a worsening of the psychological symptoms.

Threshold Issues

a. The relevant law

[121] MVA #1 and #2 are covered by the Ontario Motorists Protection Plan amendments to the Insurance Act (O.M.P.P.). Section 266(1) provides an immunity from liability for tortfeasors who cause loss or damage from the use or operation of an automobile in Canada, unless the injured person has died or sustained a certain level of impairment. This is commonly

referred to as the O.M.P.P. threshold, and I will refer to it as such throughout this judgment.

[122] The relevant section of the Insurance Act is as follows:

266(1) In respect of loss or damage arising directly or indirectly from the use or operation, after the 21st day of June, 1990, of an automobile and despite any other Act, none of the owner of an automobile, the occupants of an automobile or any person present at the incident are liable in an action in Ontario for loss or damage from bodily injury arising from such use or operation in Canada, the United States of America or any other jurisdiction designated in the No-Fault Benefits Schedule involving the automobile unless, as a result of such use or operation, the injured person has died or has sustained,

- (a) permanent serious disfigurement; or
- (b) permanent serious impairment of an important bodily function caused by continuing injury which is physical in nature.

[123] It is the plaintiffs' position that MVA #1 and #2 caused impairment to the plaintiff which exceeded the threshold set out in s. 266(1)(b). The defendants in both cases deny that the plaintiff's impairment exceeded the threshold.

[124] The case of Meyer v. Bright (1993), 15 O.R. (3d) 129, 110 D.L.R. (4th) 354 is an Ontario Court of Appeal case which sets out three questions that are to be answered sequentially in order to determine if the plaintiff's claim exceeds the threshold. At p. 137 O.R. the court set out the three questions as follows:

1. Has the injured person sustained permanent impairment of a bodily function caused by continuing injury which is physical in nature?
2. If the answer to question number 1 is yes, is the bodily

function, which is permanently impaired, an important one?

3. If the answer to question number 2 is yes, is the impairment of the important bodily function serious?

[125] MVA #3 is covered by the Bill 164 amendments to the Insurance Act. This legislation contains a different type of immunity from liability for a tortfeasor and creates a different threshold. I will refer to this as the Bill 164 threshold.

[126] Section 267.1(1) of Bill 164 also provides that the owner of an automobile, the occupants of an automobile and any persons present at the incident are not liable in a proceeding in Ontario for loss or damage from bodily injury or death arising directly or indirectly from the use or operation of the automobile in Canada. This is a blanket immunity from a finding of liability against a tortfeasor.

[127] Section 267.1(2) then provides a specific exception to this immunity if the impairment to the plaintiff meets a certain threshold. Section 267.1(2) is as follows:

267.1(2) Subsection (1) does not relieve a person from liability for damages for non-pecuniary loss, including damages for non-pecuniary loss under clause 61(2)(e) of the Family Law Act, if as a result of the use or operation of the automobile the injured person has died or has sustained,

- (a) serious disfigurement; or
- (b) serious impairment of an important physical, mental or psychological function.

[128] There are significant differences between the two thresholds. The most obvious is that the O.M.P.P. threshold requires an impairment to be permanent, whereas the Bill 164 threshold does not have any temporal restriction. Also, the O.M.P.P. threshold excludes any consideration of psychological or emotional impairment for the purpose of meeting the

threshold, whereas mental or psychological function is specifically referred to in Bill 164. Moreover, the blanket immunity from liability set out in Bill 164 includes protection against liability for both pecuniary and non-pecuniary damages, and there is no exception at all with respect to pecuniary damages. Therefore, for any motor vehicle accident which occurred while Bill 164 was in effect, the tortfeasor is not liable under any circumstances for pecuniary loss.

b. The law regarding multiple tortfeasors as it relates to threshold issues

[129] I am told by all counsel that there is no trial court or Court of Appeal decision that provides any guidance to me as to how I should handle the situation in which there are multiple tortfeasors and multiple thresholds. In fact, there are no court decisions which even consider the situation in which there are multiple tortfeasors and a single threshold. Therefore, I must formulate the appropriate test from analogy and first principles.

[130] Plaintiffs' counsel has suggested that I find that where there are multiple motor vehicle accidents that cause injuries to the plaintiff which, on a global assessment, would exceed the threshold, then every tortfeasor who caused any of the multiple MVAs should be found to have caused injuries which exceed the threshold. Plaintiffs' counsel submits that this would prevent an absurd result that would occur if none of the accidents in themselves caused the injuries which exceeded the threshold, but the plaintiff, when globally assessed, exceeded the threshold. He says that I can mollify those tortfeasors who made a minor contribution to the plaintiff's impairment when I apportion responsibility for the plaintiff's condition between the tortfeasors.

[131] I have difficulty with the plaintiffs' counsel's broad suggestion. Both O.M.P.P. and Bill 164 clearly contemplate situations in which certain tortfeasors would be immune from liability. Therefore, it would be contrary to the intention of the legislature to find liability against every tortfeasor who made even the most minor contribution to the plaintiff's

overall condition.

[132] On the other hand, defence counsel have suggested that the specific effects of each individual motor vehicle accident have to be examined and a decision has to be made with respect to the threshold on an accident-by-accident basis. This is an attractive proposition if it can be done, but in reality it is often impossible to distinguish between the effects of various MVAs on the plaintiff's condition. Where the plaintiff has suffered from the combined effect of several motor vehicle accidents, in the sense that the injuries or the effects of the injuries are intermingled, I find that the plaintiff's damages must be assessed on a global basis as the separate effect of each accident cannot be accurately determined.

[133] By analogy, I have looked to the case of *Athey v. Leonati*, [1996] 3 S.C.R. 458, 140 D.L.R. (4th) 235. This is a Supreme Court of Canada case which dealt with the situation in which a plaintiff had a pre-existing back condition and subsequently was involved in two motor vehicle accidents. The issue was whether or not the plaintiff's damages as against the tortfeasors should be reduced as a result of the pre-existing back condition. There was a single indivisible injury, a disc herniation, and it was not possible to distinguish between the effects of the pre-existing problem and the effects of the motor vehicle accidents. In that case, the Supreme Court of Canada stated that the general, but not conclusive, test for causation is the "but for" test. However, the court stated at p. 466 S.C.R., p. 239 D.L.R. that the "but for" test is unworkable in some circumstances, so the courts have recognized that causation is established where the defendant's negligence "materially contributed" to the occurrence of the injury. Also, at pp. 467-68 S.C.R., pp. 239-40 D.L.R., the court quoted with approval from the case of *McGhee v. National Coal Board*, [1972] 3 All E.R. 1008, [1973] 1 W.L.R. 1 (H.L.) as follows:

It has always been the law that a pursuer succeeds if he can shew that fault of the defender caused or materially contributed to his injury. There may have been two separate causes but it is enough if one of the causes arose from fault of the defender. The pursuer does not have to prove that this

cause would of itself have been enough to cause him injury.

[134] The *Athey v. Leonati* case has been adopted and applied in the Ontario Court of Appeal case of *Alderson v. Callaghan* (1998), 40 O.R. (3d) 136, 42 C.C.L.T. (2d) 230. In that case, at page 139 O.R. the court stated:

In light of *Athey*, supra, the jury should have been told that if Alderson's overall condition resulted from the cumulative effect of the injuries sustained in the August 10 incident, the multiple beatings inflicted upon her thereafter, and her pre-existing psychological condition, she would nonetheless be entitled to full compensation so long as the jury was satisfied, on a balance of probabilities, that the injuries sustained in the August 10 accident materially contributed to her overall condition.

[135] Applying *Athey* and *Alderson* to the case of multiple tortfeasors with a single threshold, it is my opinion that if a plaintiff suffers from injuries which, globally assessed, put his condition over the threshold, then the tortfeasors who materially contributed to the plaintiff's global condition are to be found to have caused injuries which exceeded the threshold, and therefore are not to be immune from liability by reason of the protection afforded by the Insurance Acts.

[136] This issue is further complicated by the fact that there are two separate and distinct thresholds in this case. Therefore, I rely by analogy on the case of *Hicks v. Cooper* (1973), 1 O.R. (2d) 221, 41 D.L.R. (3d) 454 (C.A.) and on the case of *Berns v. Campbell* (1974), 8 O.R. (2d) 680, 59 D.L.R. (3d) 44 (H.C.J.). These cases stand for the proposition that assessments must be made of the plaintiff's injuries at certain points in time in order to determine the plaintiff's condition for the purpose of apportioning damages between motor vehicle accidents. I am of the opinion that where there is more than one threshold involved, the plaintiff's damages must be assessed at appropriate times after each motor vehicle accident in order to determine the combined effect of the accidents up to that point. When making these assessments I am also cognizant of the case of *Hiscox v. Badger*, [1979] O.J. No. 15, which states essentially

that I should not interpret the case of Hicks v. Cooper strictly, but I should base my findings and make my assessments on all of the evidence that was before me at the trial.

[137] Therefore, in order to determine which, if any, of the motor vehicle accidents caused injuries that exceeded the threshold, where there are multiple MVAs and multiple thresholds, I find that I must go through the following exercise:

Step One: I must assess the plaintiff's condition as it existed after MVA #1 and immediately before MVA #2, and make a determination as to whether, given all of the evidence, the plaintiff's injuries exceeded the threshold that was applicable at the time of MVA #1. If so, then the tortfeasor in MVA #1 will be found to have caused injuries that exceeded the threshold that was in place at the time of MVA #1. The effects of MVA #1 should still be reconsidered in Steps Two and Three as different thresholds may apply.

Step Two: I must then assess the plaintiff's condition after MVA #2 and immediately before MVA #3, to determine whether or not his condition, globally assessed, exceeded the threshold in place at the time of MVA #2. If it does, then I need to make a finding of fact as to whether MVA#1 or MVA #2, or both, made a material contribution to the plaintiff's condition. Any tortfeasor that is found to have made a material contribution to the plaintiff's condition will be found to have caused injuries which exceeded the threshold that applied at the time of MVA #2.

Step Three: If there is a third MVA, I must again assess the plaintiff's condition globally as at the date of trial to determine whether or not the plaintiff's condition exceeded the threshold that applied at the time of MVA #3. If so, I must then make a finding of fact as to which, if any, of MVA #1, #2 or #3 made a material contribution to the plaintiff's condition after MVA #3. Those tortfeasors that made a material contribution to the plaintiff's condition will be found to have caused injuries that exceeded the threshold that applied at the time of MVA #3.

c. Analysis

[138] Applying Step One to the Baillargeon situation, I find that MVA #1 caused a serious and permanent impairment of an important bodily function, caused by continuing injury which is physical in nature. It is clear from the evidence that the plaintiff suffered from a fractured C-6 vertebra, spinal cord injury, soft tissue injury to the neck and back and an acceleration of the degenerative process in the neck and back, all as a result of MVA #1. These problems led to the cervical fusion and lumbar laminectomy. These problems are permanent and they have had a serious effect on the plaintiff's employment and daily activities. The three questions posed by Meyer v. Bright are all answered in the affirmative. Therefore, the impairment caused by MVA #1 has exceeded the O.M.P.P. threshold.

[139] In Step Two I must assess the plaintiff's condition globally after MVA #2 and immediately prior to MVA #3. I have found at that time that the plaintiff was suffering from the combined effects of MVA #1 and #2. Those combined effects were permanent and serious. Again, I answer the three questions posed by Meyer v. Bright in the affirmative. Therefore, the plaintiff at that time exceeded the O.M.P.P. threshold.

[140] MVA #1 caused the initial injuries described above. Because the threshold is the same for MVA #1 and #2 there is no need for any further finding regarding MVA #1. Also, I have already found as a fact that the injuries caused by MVA #2 were in the form of an aggravation of the injuries to the cervical and lumbar spines, and the effects of MVA #2 were significant. Therefore, I find that both MVA #1 and MVA #2 materially contributed to the plaintiff's condition as it existed immediately prior to MVA #3. Therefore, both MVA #1 and MVA #2 are found to have caused an impairment that exceeded the O.M.P.P. threshold.

[141] In Step Three, I must make a global assessment of the plaintiff's condition after MVA #3 in light of the Bill 164 threshold that existed at the time of MVA #3. I have no

hesitation in finding that the plaintiff's impairment was well in excess of the Bill 164 threshold. There is no requirement for permanency of injury under Bill 164, but the fact that the plaintiff has suffered a permanent injury can certainly be taken into account when considering whether or not the injury was "serious". There is no doubt that the plaintiff's injuries were serious after MVA #3. There is no doubt that MVA #1 and MVA #2 made material contributions. The only issue then is whether or not MVA #3 materially contributed to the plaintiff's condition after MVA #3.

[142] I have already found that MVA #3 caused an aggravation of the injuries from which the plaintiff had already been suffering regarding the spinal cord, cervical spine and lumbar spine. This aggravation was significant. The immediate reaction of the plaintiff's body to the impact in MVA #3 is evidence of that fact. I therefore find that MVA #3 materially contributed to the plaintiff's condition, and therefore MVA #3 has caused injuries which exceeded the Bill 164 threshold.

d. Conclusion regarding threshold

[143] In conclusion, I find that the tortfeasors in all three MVAs have caused injuries to the plaintiff that exceed their respective thresholds. Accordingly, subject to the other restrictions set out in the Insurance Act, all of the defendants will be held responsible for the plaintiff's damages.

General Damages

[144] I have already found that MVA #1 caused a fracture of the C-6 vertebra. I have also found that MVA #1 caused soft tissue injuries and started an acceleration of the degenerative changes in the cervical spine and lumbar spine. This process led to permanent damage to the plaintiff's spinal cord. I have found that MVA #2 and #3 aggravated all of the initial injuries. This combination of injuries and aggravations led directly to the surgeries on the cervical and lumbar spine.

[145] At the present time, John Baillargeon testified that he

still gets headaches, but they are only light headaches. He still has some light numbness in his face. He said that his neck pain significantly improved with the surgery of September 1998, but was not entirely gone. He is still working with his doctors on an area of pain at the base of his neck. Of course, his range of motion is reduced because of the fusion of the cervical spine.

[146] Mr. Baillargeon testified that he still has a little numbness in his left shoulder, radiating into his upper arm, but this was not a significant problem. He still has some numbness in both hands, and his hand occasionally cramps up or distorts, particularly if he is working on his boat models.

[147] Mr. Baillargeon testified that his low back has been feeling much better since the surgery in October 1999. He still has some discomfort but it is not like the pain he had before surgery. Again, he has some restriction in his range of motion because of the lumbar surgery. His wife assists him with cutting his toenails and putting on his socks because he cannot bend over properly. He periodically has lightness in his legs, but does not have the burning sensation and the shooting pains that he described earlier. He has had a significant improvement in the bladder control problem since the surgery. He no longer wears a pad. Both his bowel and bladder control have improved to the point where he gets some warning of his need to use a washroom, but when he has the warning, he has an urgent need to find a washroom.

[148] Regarding Mr. Baillargeon's activities, he states that he still has trouble sleeping because he is unable to find a comfortable position. This leads him to be physically and mentally fatigued. He still has trouble with his attention span and concentration, and I attribute this to the ongoing emotional and psychological stresses. Mrs. Baillargeon said that her husband continues to ramble and speak tangentially, and again I attribute this to the emotional and psychological stress.

[149] Mr. Baillargeon is unable to do most of the chores that he normally had done around the house, such as decorating,

heavy cleaning and other heavy work. His model building is restricted somewhat when his hand cramps up or his back hurts. He has difficulty playing pool because of his pain and restriction of movement in the back.

[150] Dr. Chakravarthi stated that the plaintiff's condition would not improve over time. There was a risk that there would be further degeneration of the discs immediately above and below the fusion in the cervical spine and above and below the laminectomy in the lumbar spine. This implies that the plaintiff's condition will not improve, but would likely deteriorate. I find that it is probable that the plaintiffs emotional condition will improve, assuming that he undergoes the psychological counselling that is requested in the future care costs report.

[151] On a global assessment as at the date of trial, I assess the plaintiff's general damages at \$110,000.

[152] Liability for the plaintiff's general damages must be apportioned between the three motor vehicle accidents. I do not believe that I can apply the case of Hicks v. Cooper strictly as that case would require me to assess the general damages for MVA #1 as if the damages were being assessed the day before MVA #2, and so on. Much of the causation evidence in this case did not come to light until the surgeries in 1998 and 1999. It was at that time that Dr. Chakravarthi was able to actually view the plaintiff's cervical and lumbar spine and make determinations with respect to the nature and extent of the injuries suffered at various points in time. He was also astute enough to then review the 1993 X-ray films in order to form conclusions with respect to causation. I therefore prefer the approach set out in the case of Hiscox v. Badger, supra, in which Justice Osler states at p. 6:

While much of the evidence upon which I base my findings and must make my assessment only became available subsequent to the second accident, I am persuaded that almost all of the plaintiff's pain and suffering, past, present and future disability, and loss of enjoyment of life, were caused by the first accident. I therefore assess her damages from that

accident not as I would have had the trial occurred immediately before the second accident, but as I am now able, relating subsequent events to that accident.

[153] Applying the principles of *Hicks v. Cooper*, *Berns v. Campbell and Hiscox v. Badger*, I assess general damages immediately before MVA #2 to be \$66,000. Furthermore, I assess general damages immediately before MVA #3 to be \$88,000. Therefore, by simple subtraction, general damages are assessed against each of the tortfeasors as follows:

(a) against Murray re: MVA #1--\$66,000

(b) against Kirk re: MVA #2--\$22,000

(c) against Sukdeo re: MVA #3--\$22,000

Bill 164 Deductible

[154] Section 267.1(8) of Bill 164 provides that the damages for non-pecuniary loss, other than damages under the Family Law Act, are to be reduced by a deductible, which, with indexing, is \$10,921.44. Therefore, this amount will be deducted from the general damages that are otherwise attributable to the tortfeasor in MVA #3.

Past Loss of Income

[155] The plaintiff made two attempts to return to work after MVA #2, but they were entirely unsuccessful. I find as a fact that the plaintiff has been unable to work at any gainful employment since MVA #1.

[156] Dr. Carom testified that the likelihood of the plaintiff engaging in any employment or occupation for which he was reasonably suited by reason of his education, training or experience was nil. Dr. Chakravarthi testified that it was highly unlikely that the plaintiff would return to any gainful employment. His condition is not expected to improve. I therefore find that the plaintiff will not be gainfully employed for the balance of his life.

[157] The plaintiff testified that he did not have any definite plans regarding retirement prior to the MVAs. Cecile Baillargeon testified that she and the plaintiff had never discussed the plaintiff's retirement plans. On his examination for discovery the plaintiff said that he would have probably retired by age 65, but then he added that he felt that he could not afford to do so, and therefore he would probably work beyond age 65 part-time doing something.

[158] Given the fact that Mr. Baillargeon had been working in the heavy construction industry for his entire working life, and the fact that he had pre-existing degenerative disease that was significant throughout his spine, although it was not symptomatic, and given the fact that his wife had retired early as a result of her osteoporosis, and given the fact that it was one of Mr. Baillargeon's stated intentions to spend time together with his wife during his retirement, I find that Mr. Baillargeon would not have worked part-time or otherwise after age 65.

[159] I therefore find that but for the motor vehicle accidents, the plaintiff would have worked at Marcon Custom Metals Inc., or a similar position, until he turned 65 years of age and then he would have retired absolutely.

[160] Dr. Chakravarthi testified that MVA #1 was the major cause of the plaintiff's disability which led to his inability to work. He said that it was highly unlikely that the plaintiff would have returned to any employment after MVA #1, even if MVA #2 and MVA #3 had not occurred. Therefore, I find that MVA #1 is entirely responsible for any wage loss that was suffered by the plaintiff from April 24, 1993 until the plaintiff's 65th birthday, which would occur on March 11, 2003.

[161] In this case, the maxim of "take your victim as you find him" assists the tortfeasor in MVA #2. If the plaintiff was totally disabled from employment as a result of MVA #1, then it cannot be said that MVA #2 caused any further loss of income to the plaintiff as he had no further income to lose at the time of MVA #2.

[162] Dr. Michael Charette, Ph.D., testified and provided two reports. He provided calculations with respect to lost income from April 24, 1993 to November 30, 2000. He averaged Mr. Baillargeon's earnings for the four years prior to the motor vehicle accident, then allowed for wage increases of three per cent per year from 1992 onwards in accordance with information that he obtained from Mr. Baillargeon's employer. He deducted actual earnings and disability benefits received during the period, and then he added an amount for benefits that the plaintiff would have received by way of employer pension contributions and extended health and dental benefits. I accept Dr. Charette's calculations with respect to past loss of income and find that the gross past loss of income, without any deduction for no-fault benefits, amounts to \$304,483.

Future Loss of Income

[163] I accept Dr. Charette's calculations of the present value of future loss of income to age 65 in the amount of \$97,630. In order to arrive at these calculations, Dr. Charette essentially used the same assumptions he used for past loss of income, but assumed an employment rate of 90 per cent and allowed a contingency for mortality.

[164] According to Dr. Charette's calculations, John Baillargeon has only 2.3 years left until he reaches retirement age of 65 years and the above mentioned sum represents the present value of future earnings lost over those 2.3 years. I have chosen not to apply any positive or negative contingencies to the future loss of income figure because of the very short duration of time over which the future loss of income is calculated. Also, the only obvious contingency is that of mortality, and Dr. Charette has factored that into his calculations.

No-Fault Deductibility

a. The law and the issues

[165] The Ontario Motorists Protection Plan provides that

damages awarded to a plaintiff are to be reduced by certain no-fault benefits that were or are available to the plaintiff, or to which the plaintiff is entitled. Section 267(1) of O.M.P.P. states as follows:

267(1) The damages awarded to a person in a proceeding for loss or damage arising directly or indirectly from the use or operation of an automobile shall be reduced by,

- (a) all payments that the person has received or that were or are available for statutory accident benefits and by the present value of any statutory accident benefits to which the person is entitled;
- (b) all payments that the person has received under any medical, surgical, dental, hospitalization, rehabilitation or long-term care plan or law and by the present value of such payments to which the person is entitled;
- (c) all payments that the person has received or that were or are available for loss of income under the laws of any jurisdiction or under an income continuation benefit plan and by the present value of any such payments to which the person is entitled; and

.

[166] It was agreed that the damages for past loss of income are to be reduced by the no-fault benefits actually received as weekly income benefits from the plaintiff's own insurer, Zurich Insurance Company. It was agreed that the plaintiff actually received weekly income benefits in the sum of \$110,855, and this amount should be deducted from the past loss of income of the plaintiff. It should be noted that this sum amounts to weekly income benefits paid to the plaintiff for the period commencing April 24, 1993 and ending January 2, 1996, plus a lump sum of \$50,000 which the plaintiff accepted from Zurich Insurance Company in or about January 1996, in consideration for releasing Zurich Insurance Company from liability for any

further weekly income benefit payment.

[167] The defendants take the position that the plaintiff had available to him a further sum of \$87,511 for weekly income benefits that the plaintiff should have received from Zurich Insurance Company up to the date of trial. Because the plaintiff made an improvident settlement with Zurich Insurance Company, the defendants say they should get credit for this further sum against the past lost income award pursuant to s. 267(1)(a).

[168] Similarly, the defendants state that the plaintiff was entitled to continue to receive weekly income benefits beyond the date of this trial for life. The plaintiff would have received these no-fault benefits had he not released Zurich Insurance Company from liability for any further no-fault benefits. Therefore, the defendants want credit for the amount of weekly income benefits to which the plaintiff was entitled in the future.

[169] The Ontario Court of Appeal dealt with a similar issue in the case of *Chrappa v. Ohm* (1998), 38 O.R. (3d) 651, 159 D.L.R. (4th) 215, as it related to s. 267(1)(c). In that case the plaintiff had received long-term disability benefits to the date of the trial, and was continuing to receive LTD benefits as of the date of the trial. The Ontario Court of Appeal confirmed that the benefits actually received were to be deducted from the past lost income award in the tort action.

[170] The Court of Appeal declined to reduce the future lost income award by the present value of future LTD benefits as the defendant had not proven that it was beyond dispute that the plaintiff qualified for future LTD benefits in every respect. At p. 657 O.R., the court stated:

Thus, in my view, the jurisprudence supports the view that where the concept of entitlement to future long-term insurance benefits is used as a basis for reducing the plaintiff's damage recovery it must be strictly interpreted to require that it be beyond dispute that the plaintiff qualifies for these future payments in every respect.

[171] Therefore, it has been clearly established in Ontario that there is an onus on the defendant to prove that it is beyond dispute that a plaintiff is entitled to future disability benefits in every respect in order to require the plaintiff's tort award for future lost income to be reduced by the present value of any such future benefits pursuant to s. 267(1)(c).

[172] This same onus and standard of proof also applies to a situation in which the defendant requests a reduction of the tort award for future lost income pursuant to s. 267(1)(a). The Ontario Court of Appeal confirmed that this approach applied to s. 267(1)(a) in the case of *Bannon v. McNeely* (1998), 38 O.R. (3d) 659, 159 D.L.R. (4th) 223.

[173] The Court of Appeal in *Bannon v. McNeely* attempted to further clarify the deductibility issue at p. 679 O.R., pp. 243-44 D.L.R. where the court stated:

. . . I believe that, where possible, any no-fault benefit deducted from a tort award under s. 267(1)(a) must be deducted from a head of damage or type of loss akin to that for which the no-fault benefits were intended to compensate. In other words, and employing the comparison of *Morden J. in Cox*, supra, if at all possible, apples should be deducted from apples, and oranges from oranges. It follows further from this conclusion that if the no-fault deduction exceeds the amount awarded under the specific head of damages to which the no-fault benefits can be attributed, then there cannot be resort to another portion of the tort judgment for the balance.

[174] In the case of *Dalmatin v. Raposo* (2000), 46 O.R. (3d) 749 (S.C.J.), Justice Spiegel found that future loss of income was a separate and distinct head of damages from past loss of income. Thus, no-fault benefits received or available prior to the trial are to be deducted from past loss of income, and no-fault benefits to be received in the future are to be deducted from future loss of income, subject to the defendant satisfactorily proving its right to claim the deductions to the

standard set out by the Ontario Court of Appeal.

[175] Unfortunately, there is no case which considers the standard of proof required for a reduction of past loss of income by no-fault benefits which were not actually received by the plaintiff, but which the defendants say "were or are available" to the plaintiff for a period of disability prior to the trial.

[176] Section 267(1)(a) can actually be divided into two parts; the first part states that the damages must be reduced by all no-fault benefits that "the person has received or that were or are available for statutory accident benefits . . .". I assume that this is a reference to a deduction against past lost income. The second part of 267(1)(a) states that the damages are to be reduced ". . . by the present value of any statutory accident benefits to which the person is entitled". I assume this is a reference to a deduction against future lost income.

[177] The standard of proof discussed in *Chrappa v. Ohm* was only considered by the court in relation to the second part of s. 267(1)(a). The "beyond dispute" test was only applied to future no-fault benefits which could be deducted from future loss of income.

[178] I find that the standard of proof set out in *Chrappa v. Ohm* is restricted to cases in which the defendant claims credit for no-fault benefits to which the plaintiff is entitled in the future. The court was concerned in that case about releasing the defendant completely when there was some uncertainty about the plaintiff's future entitlement to receive benefits. Section 267(1)(c) was interpreted in the aforementioned manner in order to prevent a situation in which the tort defendant was released without the plaintiff actually being entitled to receive disability benefits in the future.

[179] So, what standard of proof is required for a defendant to obtain credit against past lost income for no-fault benefits available to the plaintiff for a period of disability prior to the trial? Clearly, if the benefits were received by the

plaintiff there should be no issue of deductibility. What standard of proof is required for a defendant to show that no-fault benefits "were or are available" for a period prior to the trial in a situation in which no-fault benefits have not been received by the plaintiff?

[180] Plaintiff's counsel has urged me to apply the *Chrappa v. Ohm* standard of proof to the first part of s. 267(1)(a). He relied on the case of *Stante v. Boudreau* (1980), 29 O.R. (2d) 1, 112 D.L.R. (3d) 172 (C.A.), in which the plaintiff was not found to be "entitled" to receive no-fault benefits where the plaintiff had requested no-fault benefits and the request had been denied. In my view, the present action does not involve the same scenario.

[181] In the present case, the plaintiff was not denied no-fault benefits, but freely negotiated a settlement of his weekly income benefits with his own insurance company. The plaintiff was certainly competent and capable of making such a settlement, but he did so at his own peril. The court in *Orchover v. Wright* (1996), 28 O.R. (3d) 263, 23 M.V.R. (3d) 201 (Gen. Div.), when considering a settlement made by a plaintiff with a no-fault insurer, stated at page 267 O.R.:

However, I do not wish to be taken to say that there is anything sinister about a cash-out. There may be very valid reasons for doing so. But those recipients who are so inclined should realize that they are voluntarily assuming a risk which may eventually be decided to be to their detriment.

[182] In the *Orchover* case, which was decided prior to the Ontario Court of Appeal decisions in *Chrappa v. Ohm* and *Bannon v. McNeely*, the trial judge appears to have required the defendant to prove the plaintiff's entitlement to both past and future no-fault benefits on a balance of probabilities, without distinguishing between the two parts of s. 267(1)(a).

[183] Lastly, I look at the case of *Cattapan v. Mitchell* (1978), 27 O.R. (2d) 87, 105 D.L.R. (3d) 508 (S.C.), which considered s. 237(2) of the Insurance Act, R.S.O. 1970, c. 224,

which was the predecessor to s. 267(1). In that case, the plaintiff had accepted a lump sum in consideration of all past and future no-fault benefits which were payable to him. The court stated at page 97:

The release was not given with the privity of that person nor with his knowledge nor acquiescence and it seems unreasonable that it should adversely affect his rights to the "extent of payments available". Can the plaintiff by his own act deprive the person liable to him of all the benefits which under the statute may flow from the plaintiff's "entitlement"? I think not.

In Cattapan, there was no discussion or suggestion that the standard of proof for the deduction was anything other than a balance of probabilities.

[184] Therefore, I find that in order for the defendants in this case to require the reduction of a tort award for past loss of income by the amount of no-fault benefits available to the plaintiff prior to the trial, the defendants must prove that the no-fault benefits "were or are available" to the plaintiff on a balance of probabilities.

b. Deductions from past loss of income

[185] The \$50,000 sum which the plaintiff accepted in lieu of benefits after January 2, 1996 represents approximately 92 weeks of benefits at the rate of \$542.30 per week. Therefore, the plaintiff actually received no-fault benefits representing payment of weekly income benefits until approximately October 9, 1997. The defendants essentially want credit for weekly income benefits that should have been received from October 9, 1997 to the date of the commencement of this trial in the amount of \$87,511.

[186] I find that the defendants have provided sufficient evidence for me to find that no-fault benefits were available to the plaintiff during this period. During the period from October 1997 to November 2000, the plaintiff underwent two separate spinal surgeries; the first in September 1998

regarding the cervical spine, and the second in October 1999 regarding the lumbar spine. During the recovery period after each of the surgeries, he was so disabled that his wife needed to go into the shower with him in order to bathe him. He was left so disabled that he could not bend over to cut his toenails or put on his socks. Furthermore, there was a Functional Abilities Evaluation conducted in October 1995 which found that the plaintiff was unable at that time to perform even the duties of a sedentary occupation. This Functional Abilities Evaluation was conducted approximately two months before the plaintiff made a full and final lump sum settlement with his no-fault insurer.

[187] In order to qualify for weekly income benefits during this period, the plaintiff would have had to establish to the no-fault insurer that his injury "continuously prevents the insured from engaging in any occupation or employment for which he is reasonably suited by education, training or experience". Mr. Baillargeon's only job experience was in the heavy construction industry as a welder/fitter. He achieved a grade 12 education. Dr. Carom said that the likelihood of Mr. Baillargeon obtaining any employment for which he was reasonably suited by education, training or experience was nil. Therefore, I conclude that the defendants have proven on a balance of probabilities that weekly benefits were available to the plaintiff during this period.

[188] Therefore, pursuant to s. 267(1)(a), the past lost income award will be reduced by the sum of \$110,855 actually received by the plaintiff, and by the further sum of \$87,511 which the plaintiff had available to him.

c. Deductions from future loss of income

[189] Using the standard of proof set out in *Chrappa v. Ohm*, I find that it is beyond dispute that the plaintiff will not engage in any form of employment whatsoever between now and age 65. I therefore find that the present value of future no-fault benefits should be deducted from the future lost income award.

[190] One further issue arises. The defendants have requested

a deduction against future lost income of all no-fault benefits that the plaintiff was entitled to receive for life, which is obviously well beyond age 65 in this case. The defendants say that the no-fault benefits payable in the future are benefits that are "akin" to the future lost income head of damages, and therefore, according to the principles in *Bannon v. McNeely*, the total amount of no-fault benefits to which the plaintiff was entitled for life should be deducted. I do not accept this argument.

[191] *Bannon v. McNeely* clearly stated that, as much as possible, the no-fault benefits should be deducted from "a head of damage or type of loss akin to that for which the no-fault benefits were intended to compensate". The court in *Bannon v. McNeely* did not deal with the issue of whether or not benefits for one period of time could be deducted against an award for another period of time.

[192] I find that it would be illogical to deduct no-fault benefits payable for life from a future loss of income award that is capped at age 65 years. No-fault benefits beyond age 65 years are not the same "type of loss" akin to a future loss of income award to age 65 years. In this regard, I adopt and rely on the reasons set out by Justice Quinn in the case *Collee v. Kyriacou* (1996), 31 O.R. (3d) 558, 25 M.V.R. (3d) 288 (Gen. Div.).

[193] Therefore, the future loss of income award will be reduced by the present value of future no-fault benefits to which the plaintiff was entitled from the date of trial to age 65 years, which amounts to a reduction of \$58,596, assuming I have interpreted Professor Welland's calculations correctly.

Future Care Costs

[194] All parties acknowledge that certain future care costs are payable to the plaintiff, assuming the plaintiff's claim exceeds the relevant threshold.

[195] In Dr. Charette's first report of August 7, 2000, he calculated future care costs under Scenario I in the amount of

\$118,494. This was based upon recommendations made by Dr. Keith Hayes, Ph.D., who is a principal with the group known as Future Care Costs Associates. I accept Scenario I as being appropriate because Scenario II contemplates a move by the plaintiff to a condominium, which I find is not imminent and not reasonable.

[196] Under this head of damages the defendants also rely upon s. 267(1)(a) of the O.M.P.P. amendments to the Insurance Act as they quite correctly state that the plaintiff had available to him no-fault benefits which would have covered medical and rehabilitation expenses for 10 years commencing with the date of the first accident. Using the standard of proof set out in *Chrappa v. Ohm*, I find that it is beyond dispute that the plaintiff had available to him no-fault benefits that would have covered all of the future care costs requested up to April 23, 2003.

[197] I note that the plaintiff made a settlement with Zurich Insurance Company by accepting the sum of \$25,000 in full satisfaction of any claim for medical/rehab benefits up to April 23, 2003. The amount of the settlement is irrelevant unless the defendants suggest that the plaintiff has received more than the value of the medical/rehab expenses, but the defendants have not made that suggestion in this case. Therefore, the defendants are responsible for all of the plaintiff's reasonable future care costs related to the MVAs from April 23, 2003 for life.

[198] Dr. Charette prepared a second report dated November 24, 2000 in which he removed a few items on the consent of all parties, and calculated the present value of future care costs that would be expended after April 23, 2003. Counsel subsequently removed a couple more items on consent and provided me with the figure of \$78,465, which represents the future care costs for Scenario I after April 23, 2003 with the agreed upon items removed.

[199] Defence counsel have submitted that a few more items listed in the future care costs report should be removed or discounted. With respect to those items I find as follows:

- (i) The expense of the stair railing in the amount of \$204.53 will be removed as it will only be an expense if the plaintiff changes his residence, and the plaintiff moves to a residence without a stair railing;
- (ii) The expense for the ergonomic chair will not be removed. If there is some duplication between the ergonomic chair and an Obus Forme, the duplication is acceptable as it simply will give the plaintiff more than one place in his house in which to sit comfortably;
- (iii) There is no duplication between the heavy housekeeping service expense and the handyman service expense, and I also find that both amounts are very modest. Therefore, both amounts will remain unchanged;
- (iv) The expense for the fitness club in the amount of \$4,167.97 will be removed as the plaintiff testified that he has regularly been a member of a fitness club since he was a young man. This is not an expense that the plaintiff will incur as a result of the MVAs.
- (v) The attendant care expenses in the Future Care Costs Report are to pay for services that Mrs. Baillargeon now provides to the plaintiff for personal care. Mrs. Baillargeon now cuts the plaintiff's toenails, cuts his hair and assists him with putting on his socks. The Future Care Costs Report assesses the value of these services at \$428.42 per month. This amount seems excessive and therefore this amount will be reduced to 50 per cent of the expense listed in the report. Therefore, the present value of the future attendant care costs will be reduced by \$26,075.81.

[200] I therefore find that the plaintiff is entitled to an award for future care costs of \$48,016.69.

[201] I now have to make a decision as to who must pay these future care costs as between the defendants. I cannot find that MVA #1 alone is responsible for the future care costs as was the case for lost income. The future care costs are as a result of all of the three MVAs. Responsibility will be divided in

accordance with my apportionment of general damages. Therefore, I apportion liability for future care costs between the three accidents at 60 per cent, 20 per cent, and 20 per cent, respectively. MVA #3 is not liable for any pecuniary damages pursuant to Bill 164, and therefore the defendant Sukdeo is relieved from paying his share.

[202] Therefore, the plaintiff will receive for future care costs the sum of \$28,810 from the defendants Murray, the sum of \$9,603 from the defendant Kirk and the sum of nil from the defendant Sukdeo.

Family Law Claim of Cecile Baillargeon

[203] Mrs. Baillargeon was born on June 14, 1934 and is currently 66 years of age. She met Mr. Baillargeon in approximately 1984 and he moved in with her a few months later. They have lived together for the last 16 years. They were married in 1992. They initially lived together in Windsor, but moved to Kitchener in the late 1980s for the purposes of Mr. Baillargeon's job. Mrs. Baillargeon had been working at Reitmans in Windsor and arranged to be transferred to Reitmans in Kitchener.

[204] When Mrs. Baillargeon was working at Reitmans in Kitchener she started to have some pains and as a result underwent some testing, which resulted in a diagnosis of osteoporosis. She does not believe that she went back to work after the diagnosis. Because she was about 61 years of age at the time, she chose to retire rather than return to work.

[205] Before the motor vehicle accidents, Mrs. Baillargeon said that she and her husband were very active together. They danced regularly; Mr. Baillargeon got Mrs. Baillargeon to go to the gym with him for a couple of years; they visited Mexico for two weeks; they regularly went to Cleveland where they attended flea markets and rented a cabin on the water; they went to air shows regularly, particularly in London; and they went to dinners and shows together. Mrs. Baillargeon did not give evidence as to the social activities that she presently had with her husband, other than to say that he was not the same

person that she had married. She also said that he had pain all of the time, had difficulty bending over, and had some emotional problems. I inferred from her evidence that many of the aforementioned activities were not undertaken any longer, and had not been undertaken since the accidents.

[206] Regarding assistance that she provided to her husband, Mrs. Baillargeon said that after MVA #1 her husband could not bend over properly and she therefore assisted him by helping him put his socks on and by cutting his toenails. She also cut his hair because he could not lift his arm. After the second accident Mrs. Baillargeon again cut her husband's toenails, cut his hair and assisted him putting on his socks.

[207] After the neck surgery Mr. Baillargeon could not bend very much at all. Mrs. Baillargeon had to get into the shower with him in order to wash her husband's feet and legs for him. She also helped him change bandages on his neck brace. This lasted for a few weeks to one month after the neck surgery.

[208] After the back surgery Mrs. Baillargeon had to help the plaintiff, Mr. Baillargeon, dress and again had to assist him with bathing. This lasted for about one month after the back surgery. At the present time, Mrs. Baillargeon still assists Mr. Baillargeon with putting on his socks and cutting his toenails. She still cuts his hair for him.

[209] Emotionally, Mrs. Baillargeon feels that she has lost the companionship of the man that she married. She repeatedly stated that Mr. Baillargeon was not the man that she married. She says that he rambles on and makes no sense. People try to get away from Mr. Baillargeon and she is embarrassed by this. This is the worst problem for Mrs. Baillargeon to handle because she does not know what to do. Also, Mr. Baillargeon is short tempered and flies off the handle even at the smallest things. He is like a child having a tantrum. He did not have a temper problem before the accident. Overall, Mrs. Baillargeon feels as if she has taken on a mother role, instead of a wife role, as it relates to Mr. Baillargeon.

[210] Mrs. Baillargeon also stated that she has lost her

sexual relationship with her husband because of these accidents. Their sex life was good before MVA #1. Right after MVA #1, Mr. Baillargeon was unable to perform. This has continued to the present time. She does not talk to him much about it because she knows that it hurts him and he is a proud man. She still loves her husband, but he tests her patience.

[211] Given the above circumstances, I assess Mrs. Baillargeon's FLA claim at the sum of \$7,000. This will be apportioned between MVA #1, MVA #2, and MVA #3 at 60 per cent, 20 per cent, and 20 per cent, respectively.

[212] There is a deductible applicable to the portion of the FLA claim assessed against the tortfeasor in MVA #3 in the amount of \$5,460.72. Therefore, the amount awarded against the defendant Sukdeo will be reduced to nil.

Summary of Decision

[213] In summary, I find that the plaintiffs are entitled to damages as follows:

(1) From the defendants Murray regarding MVA #1,

(a) General Damages--\$66,000

(b) Past Lost Income--\$304,483 less \$110,855 less \$87,511
= \$106,117

(c) Future Lost Income--\$97,630 less \$58,596 = \$39,034

(d) Future Care Costs--\$48,016 x 60 per cent = \$28,810

(e) FLA Claim of Cecile Baillargeon--\$7,000 x 60 per cent
\$4,200

(2) From the defendant Kirk regarding MVA #2

(a) General Damages--\$22,000

(b) Past Lost Income--nil

(c) Future Lost Income--nil

(d) Future Care Costs--\$48,016 x 20 per cent = \$9,603

(e) FLA Claim of Cecile Baillargeon--\$7,000 x 20 per cent
= \$1,400

(3) From the defendant Sukdeo regarding MVA #3

(a) General Damages--\$22,000 less \$10,921 = \$11,079

(b) FLA Claim--nil

[214] If the parties cannot settle the issues of prejudgment interest and costs, they may make arrangements through the trial coordinator to attend before me.

[215] At the end of the trial, plaintiffs' counsel orally made a motion to amend the statement of claim by increasing the prayer for relief for general and special damages. Defence counsel could not provide any evidence of prejudice to the defendants. Accordingly, where necessary, the statement of claim will be amended by increasing the prayer for relief to the amounts awarded in this judgment.

Actions allowed.